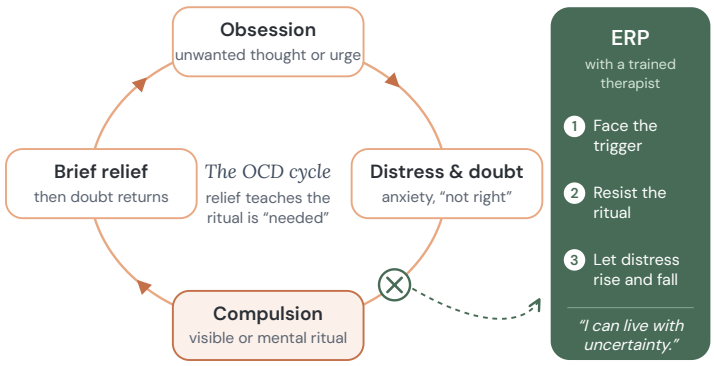
Understanding OCD: Obsessions & Compulsions

Draws on cognitive behavioral models of OCD and exposure and response prevention (ERP)

1 WHAT IS IT?

Obsessive-compulsive disorder (OCD) is a treatable condition involving obsessions (unwanted, intrusive thoughts, images, or urges that cause distress) and compulsions (actions or mental rituals done to ease that distress or prevent a feared outcome). It isn't a quirk or a love of neatness, and it doesn't reflect who you are.

2 HOW DOES IT WORK?



An original illustration of the OCD cycle and where ERP intervenes. Compulsions can be visible (washing, checking) or mental (reviewing, counting).

3 WHY DOES IT HAPPEN?

Intrusive thoughts are remarkably common: in a study across six continents, about 94% of people reported having them. In OCD, a passing thought gets treated as dangerous or as a sign of who you are, which creates intense distress and a strong pull to neutralize it.

Compulsions bring relief, so the brain learns they're necessary. But each ritual also confirms the thought was a real threat, and doubt soon returns, often needing more ritual next time.

Normal or a concern? Many people have odd intrusive thoughts or like things orderly. OCD is diagnosed when obsessions or compulsions take a lot of time (for example, over an hour a day) or cause significant distress or impairment.

4 WHAT CAN HELP?

Exposure and response prevention

ERP is a well-established first-line treatment for OCD. With a trained therapist, you gradually face triggers while resisting compulsions, so your brain learns that distress fades and uncertainty is bearable.

Relate differently to thoughts

Practice noticing, "I'm having the thought that…" A thought is a mental event, not a fact, a prediction, or a verdict on your character.

Find specialized care

Look for a clinician trained in ERP. Medication (often an SSRI) can also help, and many people use both. Reassurance, even from loved ones, can unintentionally feed the cycle.

5 REFLECTION QUESTION

What might it be like to let a thought be just a thought, without needing to settle it right now?

6 KEY TAKEAWAY

The goal isn't to stop intrusive thoughts. It's to stop letting them decide what you do.

7 REFERENCES

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These resources are for education and self-reflection and are not a substitute for individualized mental health care.

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